



SOMALIA

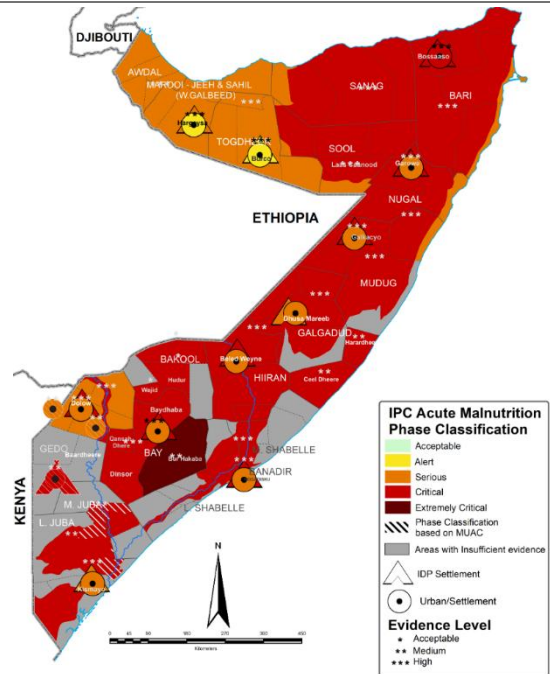
Monthly Humanitarian Update

May 2026

This report is produced by OCHA Somalia in collaboration with humanitarian partners. It provides information on the humanitarian situation across Somalia in May 2026.

HIGHLIGHTS

- Six million people are facing high levels of acute food insecurity between April and June, including 1.9 million in emergency. Nearly 1.88 million children are expected to need treatment for acute malnutrition; 42,000 more than previously projected.
- Buurhakaba district in Bay region is facing the risk of famine if a worst-case scenario develops where the current April to June rains fail, food prices continue to rise sharply, and assistance is not scaled up.
- Assistance is being scaled up amidst funding constraints. The Emergency Relief Coordinator is releasing US\$10M from the UN's Emergency Fund to reach 640,000 people with lifesaving food, nutrition, health and water assistance.
- The 2026 Humanitarian Needs and Response Plan is just 19.8 per cent funded. Due to low funding, 577,000 people were reached from January to April against a target of 2.43 million – about 24 per cent coverage.
- Disease outbreaks continue to be reported across Somalia, with increasing cases of diphtheria reported in Banadir region and Galmudug State, and Acute Watery Diarrhoea (AWD)/cholera and measles outbreaks in South West and Hirshabelle States.



Source: IPC

KEY FIGURES

6M	1.88M	24%	102K	19.8%
People facing acute food insecurity in April and May.	Children requiring treatment for acute malnutrition in 2026.	People reached against target (Jan-April) - 577K against 2.43M.	People in IPC Phase 3 and above in Buurhakaba district.	Funding received for 2026 HNRP (US\$169M of \$852 requested).

Risk of famine in Bay region as food insecurity worsens

Six million people in Somalia – 31 per cent of the analysed population – are facing high levels of acute food insecurity (IPC Phase 3 or above) between April and June 2026, including 1.9 million in Emergency (IPC Phase 4), according to the updated April–June 2026 [2026 IPC integrated food insecurity and acute malnutrition analysis](#) for Somalia. Nearly 1.88 million children are expected to require treatment for acute malnutrition in 2026 – 42,000 more than previously projected. The situation is particularly concerning in Buurhakaba district, Bay region, where agropastoral livelihoods

face the risk of famine if a worst-case scenario develops in which the April to June rains fail, food prices continue to rise sharply, and humanitarian assistance is not scaled up to reach the most vulnerable people. About 102,000 people are in IPC Phase 3 and above in the district, with more than 20 per cent facing IPC Phase 4 (Emergency). Nearly 40 per cent of children under five are acutely malnourished. Partners warn that if the below-average April–June rains end early or dry spells occur during critical crop development stages in May and June, famine thresholds may be crossed within two to three months.

Humanitarian assistance scaled up amidst funding constraints

The UN Emergency Relief Coordinator (ERC) Tom Fletcher is releasing US\$10M from UN's [Emergency Fund](#) (CERF), noting that the window to prevent famine is short. The funding will provide lifesaving food, nutrition, health and water assistance to 640,000 people. Already, the [Somalia Humanitarian Fund](#) is processing an allocation of US\$2 million to scale up ongoing interventions to reduce the risk of Buurhakaba sliding into famine conditions. On 19 May, the [Somalia Disaster Management Agency](#) (SoDMA) distributed food assistance to 2,500 vulnerable households in Burhakaba, providing essential food packages to support food-insecure communities.



Humanitarian and Government officials led by the Humanitarian Coordinator George Conway (white shirt) visiting Buurhakaba in May. Photo: OCHA/René Nijenhuis.

Access to essential water and sanitation services in Buurhakaba is extremely limited, with only 13 per cent of households having access to safe drinking water and 2.2 per cent having access to improved sanitation facilities. Following a joint mission by UNICEF, Water, Sanitation and Hygiene (WASH), Health and Nutrition teams, and representatives from the authorities to Buurhakaba, significant gaps were identified in water, nutrition, and health service delivery, particularly in areas outside the district centre. Findings indicate persistently high levels of acute malnutrition, poor water quality, and affordability challenges, while an estimated 97 priority villages continue to receive little or no humanitarian assistance. UNICEF and partners have distributed 400 hygiene kits to vulnerable households in displacement settlements, with an additional 2,000 kits planned. Chlorine supplies have been dispatched to support water treatment activities, and training provided to local water operators to improve water quality management. Nutrition interventions are being undertaken including strengthened warehouse management, and technical mentoring for frontline workers. Identified priority actions include rehabilitating the Buurhakaba water system, expanding stabilisation centre capacity, ensuring uninterrupted supplies of nutrition commodities, increasing frontline staffing, and enhancing referral and supply chain management systems.

Low funding constraining response

Humanitarian assistance remains the critical lifeline for saving lives and livelihoods and addressing acute food insecurity and malnutrition in Somalia, but the situation has been exacerbated by a major reduction in services and assistance due to severe funding constraints. As of 12 June, the 2026 Humanitarian Needs and Response Plan is just 19.8 per cent funded. From January to April, humanitarian actors reached just over 577,000 people against a target of 2.43 million – about 24 per cent coverage. In 21 priority districts, 371,839 people were reached against 1.6 million targeted; with 15 of the districts at below 25 per cent inter-cluster coverage and one with no response. At the same time, the ripple effects of conflict in the Middle East are leading to increased vulnerability. Food prices – linked to fuel price increases and maritime supply chain disruptions – have risen by up to 20 per cent, weakening household purchasing power.

Climate shocks exacerbating humanitarian conditions

Uneven rainfall, flooding, and access constraints are worsening humanitarian conditions, especially in northern regions, contributing to water shortages, poor pasture conditions, and infrastructure damage. Disrupted transport routes and rising commodity prices are further increasing vulnerabilities, underscoring the need for timely support and improved access. In Sanaag, Togdheer, and parts of Woqooyi Galbeed, uneven rainfall has led to widespread water shortages and poor pasture conditions. At the same time, heavy rains and flooding have caused significant damage to critical infrastructure, including the main tarmac road linking Bossaso and Garowe. Flash floods along key corridors, particularly in Buraan, Ceel Daahir, and Karin, have rendered roads impassable and constrained movement.

According to FAO's [Famine Early Warning Systems Network](#) (FewsNet), food security outcomes are expected to vary in agropastoral and riverine areas of southern Somalia based on rainfall performance, crop prospects, and insecurity. Emergency outcomes are projected between May and September in agropastoral areas of Hiraaan region due to widespread crop failure and insecurity. In central and northern areas, the April to June rains have been exceptionally poor. Projections show that acute food insecurity and malnutrition will likely remain very high through 2026, including due to the increased risk of flooding associated with an anticipated El Niño later in the year. Life-saving humanitarian assistance needs to be scaled up and sustained urgently, especially in hotspot areas.

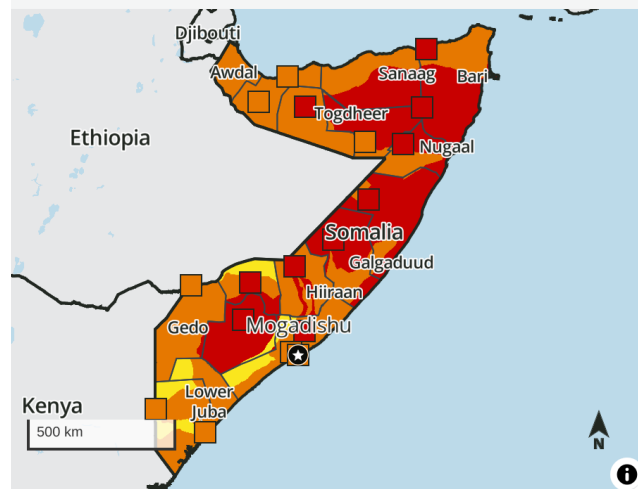
Previous drought conditions

In 2025, a prolonged dry spell triggered severe drought conditions in northern and eastern regions. The situation was worsened by the poor performance of the October to December rains, marking the fourth consecutive season of no rain or below-normal rainfall in many of the affected areas. The drought affected more than 5 million people, including more than 500,000 newly displaced people, largely pastoral and agro-pastoral households that were forced to move in search of water, food, and pasture. Losses of livestock critically undermined coping capacities and eroded resilience, while widespread crop failure was reported. The Shabelle and Juba river levels dropped sharply or dried up in some areas, severely constraining water availability. By April, the price of a 200-litre water barrel had doubled or tripled in many districts, at least 280 boreholes nationwide were non-functional, and 829 schools had been affected, with 358 closed and 471 operating under severe constraints or at risk of closure, affecting about 189,000 children.

Disease burden increasing as response dwindles

Disease outbreaks continue to present serious public health risks across multiple regions, with increasing cases of diphtheria reported in Banadir region and Galmudug, alongside persistent Acute Watery Diarrhoea (AWD)/cholera and measles outbreaks in South West and Hirshabelle States. Children and other vulnerable groups remain the most affected, with transmission driven by low vaccination coverage, weak health systems, poor water and sanitation conditions, and limited diagnostic and treatment capacity. Disease incidence is increasing at a time when health services are dwindling.

Projected area-level acute food insecurity outcomes, June - September 2026



IPC 3.1 acute food insecurity classification

Sub-national level data

- | | | |
|-------------|--------------|-----------|
| 1: Minimal | 3: Crisis | 5: Famine |
| 2: Stressed | 4: Emergency | |

Symbols

- Settlement of displaced populations
- ! Would likely be at least one phase worse without current or planned humanitarian food assistance

FEWS NET classification is IPC-compatible. IPC-compatible analysis follows key IPC protocols but does not necessarily reflect the consensus of national food security partners. For full disclosure, see endnotes.

Source: FEWS NET

More than 500 health and nutrition facilities have been closed across the country due to lack of funding, with disease outbreaks not being controlled and mortality risks rising. Total measles cases across Somalia doubled during January to March 2026 compared to the first quarter of 2025, particularly impacting vulnerable and acutely malnourished children

In Banadir, diphtheria is a major health concern, with 724 cumulative cases and 25 deaths recorded since early 2026. The region carries the highest national burden, with transmission concentrated in densely populated and vulnerable areas. Response efforts are constrained by shortages of Diphtheria Antitoxin, limited laboratory capacity, and gaps in surveillance, underscoring the need to strengthen vaccination, early detection, and community awareness.



The ERC Tom Fletcher (white shirt) visiting a nutrition center in Mogadishu in April. Photo: OCHA/Ayub Ahmed

In Galmudug, ten suspected diphtheria cases were reported in Galgaduud in the week ending 24 May, most of them in Guri Ceel. Since the start of 2026, 275 cases and four deaths have been recorded, with most cases reported from hard-to-reach areas with low immunisation coverage. Shortages of Diphtheria Antitoxin and laboratory reagents continue to limit effective outbreak control. In South West, disease outbreaks continue to affect vulnerable populations, particularly children under five, with 138 suspected AWD/cholera cases, 369 suspected measles cases, and 11 suspected diphtheria cases reported across several districts this year. Ongoing transmission has been confirmed, while limited diagnostic capacity continues to hamper timely response. The outbreak pattern reflects inadequate WASH services, low vaccination coverage, and weak health system capacity.

Story: Acute malnutrition taking a toll on children in Buurhakaba

Acute malnutrition levels have worsened substantially in Buurhakaba district, Bay region, driven by the compounding impacts of poor food consumption, high disease incidence, and reduced access to nutrition and health services. The convergence of extreme malnutrition prevalence, high levels of food insecurity, high disease burden, and severely constrained access to safe water and sanitation indicates a heightened risk of excess morbidity and mortality. The situation has been exacerbated by sharply reduced access to health and nutrition services in rural areas, with the number of treatment sites falling from 16 in 2024 to three by April 2026.

OCHA visited the Nutrition Stabilisation Centre at Buurhakaba District Hospital and found Mohamed Abdi Nurow, a 2-year-old boy from Dhafuur village, who was admitted on 7 June due to severe acute malnutrition with complications. He was brought to the hospital by his mother, Habiibo. "A prolonged drought affected our agricultural activities, significantly impacting our family livelihood," said Habiibo. "The loss of income and food production ultimately led to Mohamed's malnutrition and deteriorating health condition."



Mohamed and his mother at the centre. Photo: OCHA/Guled Issa.

Another child, Salma Siidaw Ali, 22 months old, was admitted on 4 June with severe acute malnutrition accompanied by medical complications, requiring inpatient therapeutic care. Salma comes from a highly vulnerable household with limited financial resources. "I struggle to meet basic needs and rely on irregular, unstable income," her mother, Khadijo Abdi, said. "Poverty, food insecurity, and lack of a reliable income source have placed my household at high risk, leading to Salma's deteriorating nutritional condition and complications."

Mohamed Omar, a physician at the facility, reported a rise in admissions of children with Severe Acute Malnutrition (SAM) and complications over the past two months, mostly from rural villages. From 1 January to 10 June, a total of 295 cases of Severe Acute Malnutrition (SAM) with complications were admitted to the centre, of which 65 per cent were recorded between April and 10 June, including four deaths.

Story: Rebuilding family livelihood in Buurhakaba

Shanbaqol Ibrahim Aliyow, a 40-year-old mother of five children – four girls and one boy – lives in Misaare displacement camp in Buurhakaba. In 2024, she was displaced from her village, also called Misaare, located 12 kilometres from Buurhakaba, after prolonged hardship and the loss of livelihood opportunities left her family unable to meet their basic needs.

Before displacement, Shanbaqol's family depended on menial work, including working on other people's farms and keeping livestock. They owned 10 goats and five sheep, which provided an important source of food and income. However, prolonged drought forced Shanbaqol to sell some of her livestock to cover household expenses, while others died due to the harsh conditions.

The family arrived at the displacement site after a nine-hour journey by vehicle, with very limited resources. While the journey was difficult and emotionally exhausting, life at the displacement site brought new challenges. Shanbaqol struggled to provide for her children. To survive, she borrowed money from relatives, neighbours, and local shopkeepers, but the growing debt soon became difficult to repay.



Shanbaqol taking care of her goats at Misaare displacement site in Buurhakaba. Photo: GREDO.

Registered for assistance

In January 2026, a significant development occurred when Shanbaqol was selected and registered to receive assistance through the ECHO-funded Drought Response Activation Project. She received US\$100 per month for three consecutive months, enabling her to meet urgent household needs and begin paying off her debts. "Despite the hardships we faced, I never stopped hoping for a better future for my children," she said. "Every day, I worried about how I would provide food and basic necessities for my children. When I received the cash, my situation started to change."

In partnership with [Save the Children](#), national partner [GREDO](#) is implementing a three-month drought response project through the Somalia Cash Consortium in Buurhakaba and Qansaxdheere districts. The project provides multipurpose cash assistance for 1,000 newly displaced and accessible households affected by the crisis.

"I used part of the money to pay debts, purchase food, and invest in buying seven goats," Shanbaqol said. "The goats have become an important asset for my family. They give me hope for the future and will help me to become self-reliant again. I sincerely thank GREDO and the donors for supporting me and my children during one of the most difficult periods of our lives." For Shanbaqol, the cash assistance was more than immediate relief – it was an opportunity to rebuild her livelihood, restore hope, and create a more secure future for her children.

For further information, please contact:

Ogoso, Erich Opolot | Public Information Officer, OCHA Somalia | ogoso@un.org | Tel. +254 720766587
Ayub Ahmed | Public Information Officer, OCHA Somalia | ayub.ahmed@un.org | Tel. +252 619150463